Links2Wellbeing
social prescribing for older adults

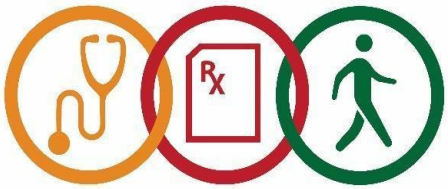
Alliance for Healthier Communities - Alliance pour des communautés en santé
Older Adult Centres' Association of Ontario - OACAO - Association des centres pour aînés de l'Ontario

Links2Wellbeing: Social Prescribing for Older Adults

Tools and Resources for Participating Centres

Phase Two 2024-2028

Updated January 2025



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Welcome

Welcome to Links2Wellbeing: Social Prescribing for Older Adults project. The Links2Wellbeing project evolved from the OACAO's Social Inclusion through Social Prescribing (SITS Rx) project funded by the New Horizons for Seniors Program and has now blossomed into a multi-year project in partnership with the Alliance for Healthier Communities with funding provided by The Waltons Trust..

On behalf of the project team, we would like to thank both continuing and new L2W partners for joining this project and for your commitment to reducing social isolation and enhancing social inclusion for older adults. We are thrilled to have you as a project partner and look forward to working with you during the second phase of the project.

Your centre's participation can be expected to reduce social isolation in older adults in your community. Programs that promote active participation by older adults have been demonstrated, time and time again, to be an effective strategy to reduce loneliness and to improve social connectedness, and mental and physical health.

We invite you to use these tools and resources as a guide to help you get started and to support you throughout the duration of the project. If you have any questions at any time, please do not hesitate to contact the project team.

For new centres joining Links2Wellbeing, as you start your training and onboarding, and for our continuing L2W partners, here are a few key items to review and/or complete:

- All up to date templates are available both in the 'Tools and Resources for Participating Centres' full print package and individually in digital format on the **Links2Wellbeing** Google Drive
- Each new participating centre must sign a Letter of Agreement, and continuing centres will be asked to sign an Addendum to the Letter of Agreement to receive additional funding.
- We want to hear from you! If you have any comments or suggestions about the project, please send an email to: L2W@oacao.org
- Your team is comprised of OACAO leadership and coordinating staff as well as expert project consultants with collaboration from the Alliance for Healthier Communities. Review our staff and consultant contact list in the google drive.

We look forward to reaching new heights by collaborating with you and your centre.

Sincerely,

Links2Wellbeing Team

SECTION A - Project overview

The **Links2Wellbeing: Social Prescribing for Older Adults** project represents a unique partnership between the Alliance for Healthier Communities and the Older Adults Centres' Association of Ontario (OACAO). The multi-year project, which began in April 2021, is funded by Waltons Trust.

The Alliance for Healthier Communities (Alliance) is a network of over 100 community-governed, team based primary healthcare organizations across Ontario. Alliance member centres are committed to supporting those who face the highest barriers to health and wellbeing, including marginalized older adults. In 2018, The Alliance brought social prescribing to Canada with Rx: Community, a made-in-Ontario approach built on the model that has revolutionized care in the United Kingdom.

The Older Adult Centres' Association of Ontario (OACAO) provides advocacy, education, networking, and resources to support a network of over 240 community-based older adult centres and organizations serving older adults across Ontario. As the voice of older adult centres and Seniors Active Living Centres (SALCs), the OACAO supports member centres to come together to unify their efforts towards the development of comprehensive programs and services, to help older adults stay connected and active in their communities.

Purpose of the Links2Wellbing project: The overall purpose is to support older adults who may face negative health impacts from social isolation and loneliness and may experience a lack of, or limited, connections to social and recreational opportunities.

What is Social Prescribing? Social prescribing is a means for healthcare providers to use a formalized pathway of referral to connect patients to a range of non-clinical programs and services in the community, to improve their social, mental, and physical health and wellbeing.

Defining Social Isolation and loneliness: Social isolation and loneliness are often used together but each have their own meaning.

Social isolation is a lack of social relationships and can be measured. A person can lack social connections in number OR be separated/excluded from connections.

Loneliness is less clear and exists as a subjective experience or feeling. This is a result of social relationships lacking in quantity, quality, or both compared to an individual's personal needs and expectations.

In addition, loneliness, can be identified as a negative health impact of social isolation. In seeking to reduce social isolation the Links2Wellbeing project aims to decrease feelings of loneliness, and other negative health impacts by enhancing social connections. We use self reported measures of these health impacts to gauge elements of project success. (see specific objective #2). The National Institute on Aging has provided a fulsome report *"Understanding Social Isolation and Loneliness Among Older Canadians and How to Address It"* if you wish to learn more on the topic.

Please feel free to contact the OACAO for additional resources.

Specific objectives:

1) To create intentional integrated health-social pathways between primary care providers (PCP's) and SALCs. A framework was developed in the first three years of the project and has provided the structured pathway as intended to further support physicians, nurse practitioners and other allied health providers to offer a model that is applicable to other primary care settings including Family Health Teams (FHTs) and solo-practice physicians. The model provides a structured pathway through which PCPs "socially prescribe" and refer older adults to a range of tailored, non-clinical services available through their local SALC or older adult centre. We continue to refine the referral process to ensure a safe and efficient pathway from health care provider to Seniors Active Living Centre.

2) To Improve understanding of non-clinical interventions on wellbeing and service utilization through data collection and evaluation.

This project builds evaluation capacity for the OACAO, the Alliance for Healthy Communities and for other organizations serving older adults across Ontario. A comprehensive evaluation framework was developed to track non-clinical interventions, not currently tracked within the healthcare system, but that are crucial to the understanding of the impact of social interventions in addressing social isolation, loneliness, and overall wellbeing. Evaluation resources and tool kits to provide ongoing feedback and quality management to the project will also be provided to partners.

The resources and tools are supported through ongoing training and onboarding of SALCs and CHCs throughout the project.

3) Develop and deploy infrastructure for provincial scaling to create supportive infrastructure and network to increase and expand social prescribing in Ontario.

In addition to an Ontario Social Prescribing Community of Practice, there is a Canadian Social Prescribing Community of Practice[1] focused on peer learning, knowledge sharing, development of transferable practices and standardized data frameworks.[2] This project supports shared learning and collaboration across sectors. Use the links above to register for each community of practice and stay connected to what is going on in the social prescribing community.

[1] Since the Canadian Social Prescribing Community of Practice (CoP) was launched by the Alliance and OACAO (project partners), the Canadian Institute for Social Prescribing (CISP) has been established. CISP and United Way BC have both joined the leadership team that plans and facilitates the Canadian CoP.
[2] Both the Ontario Social Prescribing Community of Practice and the Canadian Social Prescribing Community of Practice have been established. Visit https://www.allianceon.org/Social-Prescribing-Communities-of-Practice for more information.

Section B – Guide to Getting Started

The tools and resources contained in this package have been created for use by participating centres. **This package** includes all the forms and information that will help you be successful in the project. The information collected will contribute to the achievement of the previously referenced objectives.

**Please note that all forms and resources included in this package will also be sent as individual digital documents to participating centres.*

Steps to Setting up Links2Wellbeing at your centre.

- Select a Designated Centre Contact to implement the major aspects of Links2Wellbeing at your centre (see Appendix A)
    - Review the opportunities for support and education through the OACAO and more (see page 7).
- Review the Tools and Resources in full to understand forms and process
    - Customize your referral form
    - Have customized prescription pads printed through the OACAO
    - Familiarize yourself with accessing the L2W google drive
- Confidentiality and privacy

  Participating centres will want to ensure the confidentiality and privacy of clients' personal information. The following suggestions may help you in this regard:
    - Provide an independent email for the sole purpose of the social prescribing project with only a couple of designated people having password-protected access to the information.
    - Ensure that your fax machine (if available) is monitored by a trained Designated Centre Contact (DCC) and/or Volunteer Link Ambassador (VLA)
    - Ensure that all social prescriptions/Rx are kept in a secure, locked filing cabinet.
    - VLAs to sign a pledge and confidentiality agreement.
- Advertise for and train Volunteers (see Appendix A)
- Begin building a network of health care partners (see page 7 for tips and more information)
    - Note: your network can start small. 1-2 strong health care connections are often all you need to get started.
    - You can develop a process to reach out to additional providers as you begin to expand and/or make more community connections.
    - Volunteers are a great support in this area.
- Receive your first referral, or subsequent referrals
    - Follow the steps outlined in Section D (page 9)

Support and Education

The OACAO L2W Team regularly offer the following support, training and networking opportunities for staff and volunteers to help you with implementation and also to develop your knowledge of social prescribing. Please note that your board members and staff colleagues are also welcome to participate in the Social Prescribing Conversation Cafes and other special education opportunities to increase their knowledge and awareness about Social Prescribing and the L2W project.

- Help develop your network – contacting CHC's, FHT's and outreach
    - Support or attendance at meetings with community health partners to develop your social prescription network contact dena@oacao.org
- Bi-monthly Check-In Sessions: Support and trouble-shooting with staff and volunteers from participating centres with a focus on topics such as community outreach strategies and building your PCP network. (hosted on alternating last Monday of the month at 12 noon EST)
- Bi-monthly Social Prescribing Conversation Cafés provide an opportunity to take a 'deeper dive' into topics of interest. SALCs are invited to suggest topics for each Conversation Café. (hosted on alternating last Monday of the month at 12 noon EST)
- Education and Training opportunities for participating SALC's on topics of importance or identified training needs i.e.: Connectedness Coaching; Empowering Resilient Communities
- OACAO Links2Wellbeing staff are accessible via email at L2W@oacao.org

SECTION C – Building Your Network of Health Care Providers: Getting Started

Developing a strong network of primary care providers (PCPs), family health teams, allied health professionals and other community health care providers is key to developing referrals and the success of social prescribing. The OACAO is able to assist with initial connections, presentation and support, although your knowledge of your local area and services available will be important to the growth of your centre's network.

Connecting with Primary Care Providers (PCPs) and allied health professionals

In many communities and where available, SALCS will be matched with the Community Health Centre (CHC) serving that area. There is a structured referral pathway through which potential clients receive a social prescription from a health care provider that links the client to the SALC.

In addition, SALCs are encouraged to build their network of contacts with other PCPs and other community health care professionals. This may include Family Health Teams (FHTs), solo physicians, nurse-practitioner led clinics, community paramedics, pharmacists, social workers and hospital discharge planners.

Below you will find links to some organizations with whom you may want to connect.

- Family Health Teams (FHTs)
- Community Health Centres (CHCs)
- Nurse Practitioner-Led Clinics (NPLCs)
- Aboriginal Health Access Centres (AHACs)

You can locate these organizations in your community at these websites.

https://www.afhto.ca/
http://www.health.gov.on.ca/en/common/system/services/chc/locations.aspx
https://www.allianceon.org/nurse-practitioner-led-clinics
https://www.allianceon.org/aboriginal-health-access-centres

Here are some outreach tips to consider when building or expanding your referral source network.

- Collect your printed outreach materials – create and distribute information packages including: (*Note: many of these are also available digitally*)
  - Outreach Cover Letter (Appendix P)
  - The outreach flyer titled **Social Prescription for Older Adults: *A pathway for healthcare providers to link older adults to social and recreational opportunities*** *to promote their wellbeing'* (Appendix I) that introduces the Links2Wellbeing project to PCPs.
  - "Outreach Postcard" to PCPs (Appendix M)
  - "Outreach Postcard" for Older Adults (Appendix N)
  - Promotional Poster (Appendix O)
  - Information about your centre, i.e. centre flyer, newsletter and/or monthly activity calendars, minimum age, including your centre contact information
  - Referral form – customized with your email, phone number and fax # for PCPs to send to. (hard copy and email digital file to be integrated into EMR system)
  - Social Prescription Pads for PCP partners and allied health professionals. (order your customized Social Prescription pads through the OACAO team)
- Identify primary point of contact in the organization you hope to connect with – in most Family Health Teams (FHTs), it is an Executive Director; in Community Health Centres (CHCs), a Clinical Director.
  - Find out who the additional contacts will be in the organization for communication and follow-up: e.g. Community Navigator/Administration team / Medical Reception – generally the ones managing the fax system etc.
- Schedule opportunities to connect
  - Deliver materials in person (mileage for staff or volunteers can be expensed through project administrative fees)
  - Set up meetings (virtual or in person) to connect and discuss. Describe your centre, the Links2Wellbeing Project and its benefits as well as the process of referring clients to your centre.
  - OACAO support is available for presentations with PCPs and CHC partners.

Additional avenues for networking:

- Create awareness with community paramedics who perform friendly check- ins in some communities and are often the first point of contact for older adults who may be socially isolated.
- Connect with mental health professionals and organizations (e.g, CMHA).
- Host a virtual lunch and learn; invite current and future partner connections.
- Have VLA's drop off introductory packages to new potential connections.
- Suggested 30-second "Elevator Pitch" on Social Prescribing and Links2Wellbeing
    - *"Social prescribing is a process where healthcare providers prescribe social connections as part of a holistic health care approach. Through the Links2Wellbeing project we facilitate the referral process between health care providers and older adult centres or Seniors Active Living Centres (SALCs). The centres support each referred older adult in co-creating an activity plan that enhances their social, mental and physical health, as well as their overall well-being"*

Note: 'Older adult' is used on the general promotional flyer for Primary Care Providers (PCPs). Recognizing that there isn't a standard minimum age in older adult centres/SALCs in Ontario, it is suggested that you include the minimum age for participation in your centre (if applicable) in the centre specific information that you provide to PCPs. That information can be expected to guide PCPs when they make decisions about referrals.

SECTION D – The Referral Process

Once you have developed 1 or 2 partnerships with health care providers you will begin to receive referrals, move through the following process for all referrals:

1. Centre receives a referral
    - Assign a client code
    - Add referral to your tracking system (sample excel file offered in google drive) ****Note:** *it is important to track all referrals*; some referral information is used in reporting even if the client does not become a member or start participating in your programs.
2. Designated Centre Contact or VLA reaches out to the client (within 3 days of referral)
    - This introductory call may only be to develop interest and let the client know a volunteer will soon be in touch with them. **OR** if the client is ready, this may be an opportunity to book a centre tour and intake.
    - Complete a Personal Contact Information Form (optional), you may also use your own centre membership form if suitable.
    - Follow up with the referring source to let them know you have made contact
    - Assign further follow-up or contact to a VLA (if you have one/if required)
3. Get acquainted with the client

    This may be through the initial phone call or through an invitation to the centre for a coffee and a tour (note: subsidy can be used for transportation to initial visits if needed!)

4. Complete forms

a. **Client consent** (Appendix E) It is important to review the Client consent statement at the beginning of the intake process and 6 and 12 month follow ups. Use the script provided to ensure that all information needed to be informed is shared.

b. **Client information form** (Appendix F) This form will be used for the initial intake conversation AND for the 6 month and 12 month follow ups. The information collected corresponds to the expectations outlined in and committed to the overall project and is important to support the ongoing development and sustainability of the Links2Wellbeing project.

Complete the **INTAKE** section during the client's initial sign-on to the project. The intake section focuses on:

- Determining barriers to participation and discussing what your centre can offer to break down these barriers to support participation (subsidy support, transportation if available, peer support etc.)
- Developing an understanding of the clients interests and **co-creating** a plan to support program participation (Appendix G)

Co-creation is a key component of social prescribing. This is a collaborative process which includes creative problem solving while empowering the individual to improve their own health and wellbeing.

Keep the form in a secure location for follow-up at 6 and 12 months. Do not enter any information into Survey Monkey at this time. Printable and fillable PDF copies are available for your choice of storage method.

5. Following up

3 months (recommended)

DCC or VLAs are encouraged to contact the client for an informal check-in at 3 months.

- We suggest the VLA or DCC contact the client to ask how they are enjoying the program(s), maybe invite him/her to share what they like about what they are doing, and whether they are planning to join additional programs. The conversation isn't meant to be scripted.
- If the referred client has not yet joined a program at 3 months, revisit a conversation about potential barriers or support needs.

6 months & 12 months Follow-ups (required)

Schedule a follow up conversation (or interview) with each client at 6 and 12 months to complete the appropriate sections on the Client information form. **Each client is tracked from their date of intake**. The questions for each time period are the same

- If the client has not yet participated, follow the steps to review barriers and develop a participation plan

If the client HAS/IS participating, complete the questions related to the 6 month follow up, or 12 month follow up.

24 months (or more)

For each client who has continued participating in the Links2Wellbeing project beyond one year, please complete a Client Stories Report to demonstrate the impact of the project on the client. The Client Stories Report is available in the L2W Google Drive in a printable format. Where possible, clients can provide their own personal written response, or if needed, a staff or volunteer may transcribe. Completed reports can be scanned and emailed to L2W@oacao.org.

Questions on the Client Stories Report:

- "What is the most significant change that has happened to you since you started coming to the SALC?"
- "Why is this change significant to you?"
- "How has this change impacted your life?"

6. Enter information into Survey Monkey.

Once the Client Information Form is completed in full – including Intake, 6 month outcomes and 12 month outcomes, please enter the data into Survey Monkey using the link provided for L2W project reporting.

7. Discontinuing a client

- in some cases, you might reach out at the 6 or 12 month mark and either be unable to get a hold of the client or their situation has changed and they are no longer in a position to participate (in some cases it may be appropriate to revisit the barriers conversation, and in other cases your discretion may tell you that it is not, and that the client should be discharged from the project)

- complete the discontinued portion of the Client Information Form
- you may enter client data into Survey Monkey at the time of discontinuation (even if 6 or 12 month data is incomplete)

Discharging a Client/Participation beyond one year

In most cases L2W clients will remain in the program for at least 12 months. At that point, they can be transitioned to general membership status at your centre, which means they are no longer receiving a subsidy through L2W. This frees up L2W funds for new referral clients to benefit from the subsidies.

Please note, if you have sufficient subsidy funds available or under special circumstances, clients can be offered a 2nd year on the L2W project. We would ask that you please do a follow-up with them at the end of the 2nd 12-month period (24 months in the project).

SECTION E: L2W Micro-Grant - Use of Funding Guidelines

Project Funding: HUB Centres and Group A Core Centres

Project Funding: Group A: Core Centres
Micro-grant funding for new centres: funding is $2,750 per SALC/Participating Centre. This includes:

max. of $2,000 for Project Subsidies,
max. of $750 for administrative costs

Micro-grant funding for continuing centres: funding of $2,500 per year, SALC/Participating Centre is available. This includes:

max. of $2,000 for Project Subsidies,
max. of $500 for administrative costs
**submission of reporting tools and financial reports from previous years required prior to release of continuing yearly funds

Additional funding may be available on an as-needed basis for both new and continuing centres. Please contact Links2Wellbeing staff to learn more about this process.

Use of Project Funds

Subsidy funding

- may only be used for the exclusive purpose of subsidies for Links2Wellbeing client participants such as
  - centre/club membership fees; program, event and activity fees
  - transportation costs
  - program material costs.
  - other program-related expenses as approved by the OACAO

Administrative funding:

- Available to cover the cost of L2W Program Outreach and Development, Volunteer Training and Project Materials such as
  - the cost for outreach materials (e.g., printing and postage),
  - volunteer screening and training costs (i.e. vulnerable sector screening), and VLA training costs (e.g., meeting refreshments and small honorariums for volunteers).
  - Mileage costs to distribute L2W outreach packages to potential health care partners.
  - Please contact the OACAO for specific details.

HUB Centres

HUB centres will manage their own funding as described above, as well as additional funding for Group B Satellite Centres. Funding held in trust for Group B centres will be determined and confirmed in collaboration with the OACAO, HUB centre and their connected Satellite Centres.
Funds will be allocated as needed to HUB Centres to support Satellite centres in their area

Project Funding: Group B Satellite Centres

Satellite centres will not directly hold Links2Wellbeing funding and will receive subsidy support, on an as needed basis, through the HUB Centre they are connected with.

Additional information for all participating centres

Limitations of funding:

- No individuals involved will financially benefit personally from the project.
- Grant funds are not to be used by or for any organization or individual other than expenses specified in signed Letter of Agreement.
- Funds shall not support health care related expenses (ex: foot care)
- Funds shall not support programs/activities hosted by outside organizations.
- All project funds must aim to be expended during the time frame specified.

Financial Accountability:

- An annual Financial Report is to be completed each year by the due dates outlined in Section E, outlining funds spent both for subsidy and administrative costs, identifying any unspent funds. SALCs/Participating centres can request approval to carry forward unspent funds to the next project year.

Carry Forward Funding:

Unspent project funds must be reported on the financial report at each year end (Appendix L) and can be requested to carry forward to the following project year. Carry Forward funds must be approved by the OACAO. Carry forward funds must be spent prior to any additional funds being issued in the following year.

If you intend to carry forward funds, it is important to submit all year-end documentation and reports on time.

SECTION F: Appendices, Google Drive and Reporting

Appendices:

The full document and appendices, listed on page 2 under "Contents", including forms that participating centres will be required to complete as part of the Links2Wellbeing project, as well as other resources (e.g. general outreach flyer for PCPs, position description for VLAs) are available using the link below.

All of the Links2Wellbeing forms and resources are available on the OACAO Links2Wellbeing Google Drive.

https://drive.google.com/drive/folders/1Q1Ytrq6GhzH0arXEeRZlf8puEju95eC6?usp=sharing

Individual Check In's:

Participating centres will be asked to participate in a once yearly check-in meeting with the OACAO, team over Zoom scheduled at a mutually selected time. These meetings will review project progress at your centre, any needs or support required and any updates from the OACAO

Reporting Documents

Financial Reporting

Financial reports are due each year to reflect spending of subsidy and administrative funding. Reports will reflect both carry forward and any new funding received in the reporting year. See **Appendix L** for a sample report.

Common Tracking Tool

The common tracking tool tracks important data to support the OACAO's reporting to the project funder as well as ongoing sustainability initiatives and advocacy work. See **Appendix H** for the report sample.

A printable format of the Common Tracking tool is available as it may be helpful to use this throughout the year to guide your ongoing tracking.

Final Common Tracking Tool Data will be submitted via Survey Monkey. The link will be provided via email prior to the due date.

A sample excel tracking sheet, with calculations can be found in the google drive. Utilizing this sheet will ensure that you have many of the answers to complete the Common Tracking Tool at your fingertips.

Reports (Financial and Common Tracking Tool are due on:

April 15, 2025 (inclusive of April 1, 2024 through March 31, 2025)
April 15, 2026 (inclusive of April 1, 2025 through March 31, 2026)
April 15, 2027 (inclusive of April 1, 2026 through March 31, 2027)
April 14, 2028 (inclusive of April 1, 2027 through March 31, 2028)

At times the OACAO may request that your centre participate in other evaluation activities such as client self reported surveys/questionnaires, focus group discussions, research collaborations with post secondary institutions or other feedback.

Client Information Form: Data is reported through Survey Monkey when a client completes the 12 month outcomes portion of the tracking form OR is discontinued. Please ensure that all completed Client Tracking surveys for each year are entered into survey monkey by the above listed dates: **https://www.surveymonkey.com/r/D7MZG9L**

Note: we recommend that you enter the data into Survey Monkey as the 12 month outcome forms are completed, rather than wait until the end of the year to submit all forms at one time.

Appendix A – Roles of DCC, VLA and Volunteer Peer Mentors

Designated Centre Contacts (DCC)

- Community outreach: responsible for promoting the Links2Wellbeing project in the community and specifically with Primary Healthcare Providers (PCPs) and other community health care professionals (See Section B)
- Ensure all VLA's are trained and have signed the confidentiality agreement (Appendix K)
- Receive referrals from PCPs, usually by email or fax. The telephone is an option depending on the preference of the referral source (PCP)
- The DCC should confirm that initial contact has been made with the client by connecting with the referring source via fax or email.
- Ensure the information on the *Client Information Form (Appendix F)* is entered into SurveyMonkey (SM) once the 12 month outcomes or discontinued sections (which ever comes first) is complete (SM link is noted on the form)
- Ensure that tracking for the project is ongoing and reporting is completed including.
    - *Common Tracking Tool (Appendix H)*
    - *Financial Reporting (Appendix L)*
- Attend optional check-in meetings, Conversation Cafes and L2W training sessions as needed.

Volunteer Link Ambassadors (VLA) – See sample Role Posting in Appendix J

Responsibilities of VLAs

- Support the Designated Centre Contact with the intake of new referred Links2Wellbeing Participants
- Contacts referred clients, completes centre tours and intake forms
- Works with referred clients to co-create a participation plan suitable for their individual needs and interests in centre programs
- Completes follow-up surveys and data entry
- Securely handles client personal information
- May assist with reporting and referral tracking

Volunteer Peer Mentors - See sample Role Posting in Appendix J

Mentors support Links2Wellbeing referred clients in their first experiences at the centre. This role does not take on survey, reporting or intake requirements like the VLA, but supports clients in other ways.

Centres may choose to use this role of Volunteer Peer Mentors in addition to a VLA for

- Introductory tours of the centre
- Sharing centre program and participation information
- Introducing new members to class instructors and peers
- Attending a class (or two) with the new member so they feel comfortable and welcome (fees can be covered by the L2W subsidy fund)

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Appendix B **Client Referral Form**

Date:_____

Name of client:_____

Client phone number:_____

Client email address (if applicable):_____

Reason for referral. Please check all that apply.

- [ ] Social isolation
- [ ] Loneliness
- [ ] Mild[3] depression and/or anxiety
- [ ] Other. Please specify. _____

Referral source:

- [ ] Community Health Centre (CHC). Please specify role of referring individual (e.g. physician, nurse)._____
- [ ] Family Health Team (FHT). Please specify role of referring individual (e.g. physician, nurse)_____
- [ ] Community Paramedic
- [ ] Solo physician
- [ ] Nurse Practitioner-Led Clinic (NPLC)
- [ ] Hospital. Please specify department. E.g. discharge planning
- [ ] Other. Please specify.

Signature:_____ Date:_____

Please send this referral form to …
INSERT CENTRE'S fax number (as available), phone and email HERE

[3] Please note that the social and recreational programs are not able to serve clients requiring clinical interventions or those individuals who are living with significant cognitive impairments.

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INSERT CENTRE'S LOGO ***(Customizable - not actual size)***

Appendix C - 1

SOCIAL PRESCRIPTION

Rx

for:_____

Date: _____

For more information or to connect with your local centre:

call <INSERT CENTRE'S PHONE NUMBER>

or email <INSERT CENTRE'S EMAIL ADDRESS>

Signature: _____ Date: _____

INSERT CENTRE'S LOGO

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(Customizable - Not actual size)

Appendix C – 2

SOCIAL PRESCRIPTION

Rx

For: _____

Date: _____

☐ Social Activities

☐ Physical Activities and Sports/Recreation

☐ Arts and Crafts

☐ Cards and Games

☐ Educational Activities

☐ Health and Wellness

☐ Dining Activities

☐ Other: _____

For more information or to connect with your local centre:

call <INSERT CENTRE'S PHONE NUMBER>

or email <INSERT CENTRE'S EMAIL ADDRESS>

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Appendix D **Personal Contact Information Form**

Collected by Designated Centre Contact (DCC) upon first contact with referred client, and stored in a secure location.

Client Name: _____

Assigned Links2Wellbeing client code:_____

(To protect client's privacy, each client should be assigned a unique Links2Wellbeing client code at intake e.g. Get Together Seniors' Centre – GTSC, plus a unique number for each client to be assigned and used at follow up interviews and entered into Survey Monkey = GTSC1, GTSC2, et cetera.)

Home phone number:_____

Cell phone number:_____

Email address (if applicable):_____

What is your preferred method of contact?_____

Date of birth (optional)_____

Mailing Address:

_____

_____

Emergency contact information:

Name:_____ Phone number:_____

~~~
~~~

Appendix E

Suggested Script for Required Client Consent

Intake:
My name is…….I am a volunteer (*or* staff member) with the Links2Wellbeing: Social Prescribing for Older Adults Project at the [*insert name of centre* ……]. Links2Wellbeing is a collaboration between health care providers and local seniors centres and seeks to connect older adults to social activities and connections with the goal of whole person wellness. We are partnering with the OACAO to conduct an evaluation of the Links2Wellbeing Social Prescribing program to see how participating at the centre impacts your health and wellbeing.

Before we talk about your program interests and what we offer at the centre, I have a few questions to ask that will help me to get to know you better. Answering the questions is totally voluntary, and you can skip any questions you do not want to answer. Any answers you do provide are confidential and will be kept in a secure location.

Your responses will be anonymously combined with responses from other people across the province who participate in this project. Group results will be included in annual reports to the anonymous funder and partner organizations of this project. Individual identifying information will not be shared.
Your participation today will have no impact on your membership at *[insert name of centre …….]* or your participation in our Links2Wellbeing program.
Please note that some of these questions will be repeated at 6 and 12 months.

6 or 12 month follow up

My name is…….I am a volunteer (*or* staff member) with the Links2Wellbeing: Social Prescribing for Older Adults Project at the [*insert name of centre* ……]. When you started at the centre we described the Links2Wellbeing project - a collaboration between health care providers and local seniors centres which seeks to connect older adults to social activities and connections with the goal of whole person wellness. We also asked a few questions about your program interests and your social connections.

We would like to learn more about how the programs you have been participating in have impacted your health and wellbeing by reviewing these questions again. Answering the questions is totally voluntary, and you can skip any questions you do not want to answer. Any answers you do provide, are confidential and will be kept in a secure location.

We are partnering with the OACAO to conduct this evaluation. Your responses will be anonymously combined with responses from other people across the province who participate in this project. Group results will be included in annual reports to the anonymous funder and partner organizations of this project.
Your participation today will have no impact on your membership at *[insert name of centre …….]* or your participation in our Links2Wellbeing program.

Links2Wellbeing
social prescribing for older adults

Client Consent Form

Links2Wellbeing is a collaboration between health care providers and local seniors centres and seeks to connect older adults to social activities and connections with the goal of whole person wellness. We are partnering with the OACAO to conduct an evaluation of the Links2Wellbeing Social Prescribing program to see how participating at the centre impacts your health and wellbeing.

Your responses to the questions will be combined with responses from other people across the province who participate in this project. Group results will be included in annual reports to the anonymous funder and partner organizations. Your participation today will have no impact on your membership at *[insert name of centre…….]* or your participation in our Links2Wellbeing program.

Do you have any questions about the survey, or about our Links2Wellbeing Project?

Name of Participant: _____

Links2Wellbeing client code: _____

Name of Centre: _____

By signing this form, I confirm that:

- The project and its purpose have been explained to me/the participant named above.
- All questions asked by myself/the participant have been answered.

Do you agree to participate in this survey?

Intake - confirmed yes ☐
Signature of client (if in person) or VLA/DCC (if over phone) _____

6 months - confirmed yes ☐
Signature of client (if in person) or VLA/DCC (if over phone) _____

12 months - confirmed yes ☐
Signature of client (if in person) or VLA/DCC (if over phone) _____

Name of Person Obtaining Consent: _____
Signature of Person Obtaining Consent: _____

**This document is to be kept with the Client Tracking Form to be reviewed and signed at each survey date*

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Appendix F

Client Information Form

Note to Volunteer Link Ambassador (VLA) or Designated Centre Contact (DCC): Please ensure that the client consent form is completed and reviewed before each section of this tracking form.

This form is meant to "follow" your Links2Wellbeing Clients through their first year at your centre. Once you have completed the form in full (OR the client is discontinued. Whichever comes first), please input the data into survey monkey using the link at the end of the form.

SECTION A – Intake

Intake Date: _____

Client Name:_____

Client Code:_____

Home phone number:_____

Name of person completing form (VLA or DCC):

_____

Name of Centre: _____

Consent Complete ☐

1. What age group do you belong to?

☐ Under 55
☐ 55 - 64
☐ 65 – 74
☐ 75 - 84
☐ 85+
☐ Prefer not to say

2. What gender do you identify with?

☐ Male
☐ Female
☐ Identify as other than male or female
☐ Prefer not to say

3. **What are your preferred pronouns?**

☐ She/her
☐ He/him
☐ They/them
☐ Prefer not to say
☐ Other

4. **Do you identify with or belong to one or more of the following groups? Please check all that apply.**

☐ Francophone
☐ 2SLGBTQIA+
☐ Indigenous
☐ A minority group
☐ Live in a rural area
☐ Live Alone
☐ Caregiver
☐ None of the above
☐ Newcomer (arrived in Canada in the last 5 years)
☐ Settled immigrant (have lived in Canada more than 5 years)
☐ Living with a physical disability
☐ Living with a mild cognitive impairment
☐ Living with a developmental disability
☐ Living with a mental health condition
☐ Prefer not to say

5. **Do you currently receive the Guaranteed Income Supplement (GIS) from the Government of Canada or Ontario Guaranteed Annual Income System (GAINS)**?

☐ Yes ☐ No ☐ Unsure

6. **Are you currently involved in social activities (such as hobbies you enjoy, visiting with friends, or other recreational activities)?**

☐ Yes ☐ No

7. **What barriers (if any) might you face participating in programs at the centre. Please check all that apply.**

☐ Language
☐ Transportation
☐ Caregiving/family responsibilities
☐ Cost / Fees (if applicable)
☐ Physical Accessibility (e.g., of building and/or programs, mobility concerns)
☐ Hearing and/or vision impairments
☐ Declining and/or poor health
☐ Hesitant to participate
☐ Cultural practices
☐ Other

☐ None of the Above

8. Overcoming barriers

This is an opportunity to brainstorm solutions with older adult, develop a relationship through conversation, and further share what supports your centre can offer. Take this opportunity to:

- ✓ ask any additional questions related to barriers
- ✓ describe services that your centre offers (For example, if a client is hesitant to participate, a 'companion' might be able to attend a program with them for support.)
- ✓ build trust within your conversation using active listening and appreciative inquiry

Use the space below to take any notes that may be helpful for your future reference.

_____

_____

_____

---

9. Social Connection Questions

Answers to the questions below are tracked and documented during the intake conversation and again at 6 and 12 months.

****Note. The reason for tracking this is to determine whether there are changes during the time which the older adult participates in programs at the centre. For example, does the client feel less isolated (question #3) after they have been***

How often do you feel that you lack companionship?

☐ Hardly ever ☐ Some of the Time ☐ Often

How often do you feel left out?

☐ Hardly ever ☐ Some of the Time ☐ Often

How often do you feel isolated from others?

☐ Hardly ever ☐ Some of the Time ☐ Often

10. Co-creation and program selection.

Note: Please advise the client that you are co-creating the plan together with them. It is important to empower the client and for him/her/they to know that it is their interests that will guide the next steps. You are encouraged to talk about the client goals (if any) for participation at the SALC. For example, are they interested in building new skills, socializing with others, making new friends? Ask about current interests, current social connections if any (neighbors, church etc)

To assist with additional conversation, please refer to the **Leisure interests: guiding questions** (Appendix G) found in the 'Tools and Resources for Participating Centres' document.

If the client is not yet ready to choose a program(s), you may choose to provide them with information about available programs and activities. You can provide them with hardcopy program information or send them a link to the website, if applicable.

What programs has the client expressed interest in or registered for?

_____

_____

_____

---

11. Did this client receive a subsidy?

☐ Yes ☐ No ☐ Unsure

End of intake

Section B – Client Participation and Outcomes

To be completed at 6 and 12 months

6 month date for follow up _____ *consent reviewed* ☐

*12 month date for follow up*_____ *consent reviewed* ☐

Please complete ***review of client consent form*** *before beginning this survey.*

If client discontinued / you are unable to contact, no consent is needed. Move directly to Section C

In this section, at the 6 month follow up, complete answers in the sections marked 6 months. At the 12 month follow up you will ask the questions again, and complete the answer sections marked 12 months.

1. **Are you participating in programs?**

6 months
☐ Yes ☐ No

12 months
☐ Yes ☐ No

If **YES** go to Q3, if **NO** go to Q2

2. **What factors have prevented you from participating in programs? Please check all that apply.**

6 months

☐ Language
☐ Transportation
☐ Caregiving/family responsibilities
☐ Cost / Fees (if applicable)
☐ Accessibility (e.g., of building and/or programs, mobility concerns)
☐ Hearing and/or vision impairments
☐ Declining and/or poor health
☐ Hesitant to participate
☐ Cultural practises
☐ Other

12 months

☐ Language
☐ Transportation
☐ Caregiving/family responsibilities
☐ Cost / Fees (if applicable)
☐ Accessibility (e.g., of building and/or programs, mobility concerns)
☐ Hearing and/or vision impairments
☐ Declining and/or poor health
☐ Hesitant to participate
☐ Cultural practises
☐ Other

Once Q2 is complete: If appropriate, refer to Section A – Q8: Overcoming Barriers and Q10: Co-Creation and Program Selection to re-establish participant needs and interests. Determine whether any new barriers can be overcome and/or whether there are change to program offerings that might be more suitable for their interests.

3. On average, how many programs do you attend in each month? (estimate across all the programs you attend) (e.g. attend euchre 4 times/month & bingo 8 times/month = 12 programs / month = select 9 – 12 programs per month, mark with "X")

6 months

☐ 1-4 programs per month
☐ 5-8 programs per month
☐ 9-12 programs per month
☐ 13+ programs per month

12 months

☐ 1-4 programs per month
☐ 5-8 programs per month
☐ 9-12 programs per month
☐ 13+ programs per month

Now that you are participating at the centre, we have a few questions we would like to ask you about your experience.

4. Do you feel your participation at the centre has resulted in fewer appointments with your primary care provider (e.g. doctor, nurse practitioner)?

6 months

☐ Yes ☐ No ☐ Unsure

12 months

☐ Yes ☐ No ☐ Unsure

5. Do you feel your participation at the centre has resulted in fewer visits to an emergency care clinic?

6 months

☐ Yes ☐ No ☐ Unsure

12 months

☐ Yes ☐ No ☐ Unsure

6. How often do you feel that you lack companionship?

6 months

☐ Hardly ever ☐ Some of the Time ☐ Often

12 months

☐ Hardly ever ☐ Some of the Time ☐ Often

7. How often do you feel left out?

6 months

☐ Hardly ever ☐ Some of the Time ☐ Often

12 months

☐ Hardly ever ☐ Some of the Time ☐ Often

8. How often do you feel isolated from others?

6 months

☐ Hardly ever ☐ Some of the Time ☐ Often

12 months

☐ Hardly ever ☐ Some of the Time ☐ Often

Section C - Discontinued

Date of discontinuation: _____

If date is unknown, please note the closest date possible.

If known, please identify the reason for the client discontinuing their participation.

Reason for discontinuation: _____

_____

When this form has been completed in full (including intake, 6 & 12 month outcomes and/or discontinuation please input data into Survey monkey through this link **https://www.surveymonkey.com/r/D7MZG9L**

Please contact the OACAO team at L2W@oacao.org or 1-866-835-7693 if you have any questions or would like some assistance.

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Appendix G **Leisure interests: guiding questions**

Date: _____

Links2Wellbeing Client Code_____

Note to Volunteer Link Ambassador (VLA) or Designated Centre Contacts (DCCs), the categories of activities listed in the first 6 questions below correspond to those in the Record of Program Participation found in the Client Intake Form. This leisure guide provides you with a way to explore the client's interests. You can then insert the number and types of programs the client chooses on the Client Intake Form.

1) Please tell me about what types of social activities or special events you enjoy. (examples include; online movie groups, social teas, special events)

2) Please tell me about what type of physical activities and sport/recreation programs you enjoy. (examples include; line dance, yoga, fitness, sports)

3) Please tell me about what types of arts and crafts activities you enjoy. (examples include; sewing, knitting, woodworking)

4) Tell me about the types of cards and games you enjoy or have played in the past. (examples include; bingo, cards games, board games)

5) If you enjoy participating in educational activities, please tell me about ones that may interest you. (examples include: computer classes, lunch and learn, languages, virtual webinars)

6) Please describe any health and wellness programs and services that you enjoy or would like to try? (examples include; foot care clinics, nutritional programs, stress management)

7) Can you tell me what types of outdoor activities you enjoy or would like to explore? (examples include; gardening, walking, picnics, biking, pickle ball)

8) Do you have any other hobbies or interests you enjoy or do have any personal goals/something new you would like to try?

THANK YOU!

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Appendix H – Common Tracking Tool

Organization Name: _____
Contact Name: _____
Email Address: _____
Phone Number: _____ Date: _____

Links2Wellbeing – Social Prescribing for Older Adults
Common Tracking Tool* – *April 1, 2024 - March 31, 2025
Information to be completed and submitted to the OACAO by April 15, 2025
Check-ins will be scheduled with OACAO (approx. 2 per year)

Please note: *this document is a sample intended for your own notes, tracking and survey preparation. Please submit final responses to Survey Monkey -* ***https://www.surveymonkey.com/r/L2W2025***

Referral Data	
1.1 Number of Referrals received from:	**Number**
Community Health Centres (CHC):	
Family Health Teams (FHT):	
Solo Physicians:	
Community Paramedics:	
Nurse Practitioner Lead Clinic:	
Mental Health Worker: (Social worker, Case Manager, Psychotherapist etc)	
Pharmacist:	
Hospital:	
Other (Physiotherapist, Dietician etc):	
Total number of all new referrals:	
1.2 Breakdown of Referral *Sources by name(s)*	**Name(s)**
Name of referring CHC(s):	
Name of referring FHT(s):	
Name of referring Community Paramedics (e.g.: by region or municipality):	
Name of Hospital providing referrals:	
1.3 Breakdown of Referral *Sources by number*	**Number**
Number of Solo Physicians providing referrals:	
Number of Mental Health practitioners providing referrals:	
Number of Pharmacists providing referrals:	
Other:	
If other please describe:	
1.4 Potential referral partners: Number of health care providers you reached out to regarding the Links2Wellbeing Program but have received no referrals from yet (provided materials, met with to discuss the project, etc.)	

(reminder: all questions reflect April 1, 2024 - March 31, 2025)

2. Reason for Referral	
2.1 Number of referrals for loneliness	
2.2 Number of referrals for social isolation	
2.3 Number of referrals for mild depression or anxiety	
2.4 Number of referrals for other reasons (e.g. Caregiver support, food security, physical activities)	

3. Referral Outcomes	
3.1 Number of referrals that resulted in participation at SALC (i.e. became official L2W clients)	
3.2 Number of referrals that were not interested in the SALC (e.g. did not respond to initial contact, did not want to move forwards with the referral)	
3.3 Number of L2W clients that have dropped out or stopped attending the SALC	
3.4 Number of new referrals receiving subsidy	
3.5 Number of ongoing (>1 year) L2W clients receiving subsidy (i.e., those clients who began participation prior to April 2024)	
3.6 What is the average amount a L2W client is being subsidised?	

NEW **4. Human Resources**	
4.1 Total number of staff members involved in supporting L2W	
4.2 Approximate number of hours staff spent engaged in Links2Wellbeing project work (i.e.: intakes, volunteer recruitment, training and management, client support, administrative work, relationship management with health partners)	
4.3 Total number of Volunteer Link Ambassadors (VLA) engaged during the reporting year	
4.4 Number of other volunteers involved in your Links2Wellbeing program (ex: peer mentors) for the reporting year	
4.5 Total number of hours contributed by all volunteers	
4.6 Total number of students involved in supporting Links2Wellbing (e.g. practicum students)	
What programs are these students enrolled in? (e.g. Nursing, Social Work etc):	
What level of study are they completing? (college diploma, college applied degree, university undergraduate degree, graduate degree):	
4.7 Total number of post-secondary institutions partnering on project (e.g. through student placements, research projects, etc.)	
Please name above noted post-secondary institutions:	

5. Additional Questions and Feedback

5.1 What barriers or limitations, if any, have you faced in implementing your social prescribing program? What have you done to overcome these barriers?

5.2 What strengths or assets have you leveraged to be successful in the Links2Wellbeing program? Please select all that apply.

✓	**Strength/Asset**	**Please describe:**
	Grant funding for social prescribing	
	Funding from other sources (e.g.: monetary donations)	
	In-kind resources	
	Partnerships with health care providers/ community organizations	
	Buy-in from your board of directors or other senior leadership	
	Volunteer support	
	Staff buy-in to support program vision	
	Staff time allocated for social prescribing	
	Other	

5.3 What resources or supports would help social prescribing be sustainable at your centre?

5.4 Do you have other feedback about the Links2Wellbeing social prescribing project that you would like to share with the OACAO team? If so, please describe.

Thank you for completing this Common Tracking Tool.

Due date: April 15, 2025
Please enter responses into survey monkey. https://www.surveymonkey.com/r/L2W2025

If you have questions or concerns about any of the information requested above please contact L2W@oacao.org prior to the due date.

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Older Adult Centres' Association of Ontario - OACAO - Association des centres pour aînés de l'Ontario

Appendix I Social Prescriptions for Older Adults[4]

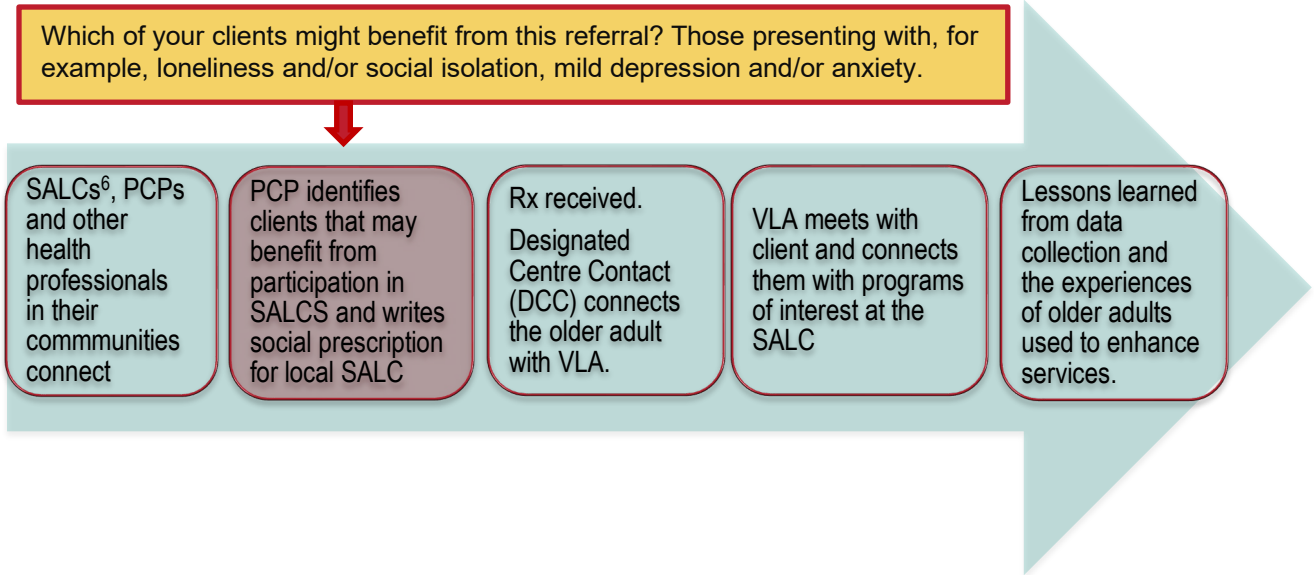
A pathway for healthcare providers to link older adults to social and recreational opportunities to promote their wellbeing

Research has shown that participating in social and recreational activities has a measurable positive effect on the health and wellbeing of older adults. Finding meaningful ways to include isolated older adults is critical; social prescriptions can help to achieve that outcome.

What is social prescribing?

Social prescribing is 'a means for trusted individuals in clinical and community settings to identify that a person has non-medical, health-related social needs and to subsequently connect them to non-clinical supports and services within the community by co-producing a social prescription – a non-medical prescription, to improve health and wellbeing and to strengthen community connections.[5]

What will social prescribing look like in your community?



[4] The Links2Wellbeing 3-year project (launched in 2021) represents a unique partnership between the Alliance for Healthier Communities and the Older Adult Centres' Association of Ontario (OACAO). For more information, please contact the OACAO at 905-584-8125 or 1-866-835-7693 or at oacao.org.

[5] Muhl, C., Mulligan, K., Bayoumi, I., Ashcroft, R., & Godfrey, C. (2022). Establishing internationally accepted conceptual and operational definitions of social prescribing through expert consensus: A Delphi study. medRxiv. https://doi.org/10.1101/2022.11.14.22282098

[6] SALC – Seniors Active Living Centres; PCP – Primary Care Provider; DCC – Designated Centre Contact; VLA – Volunteer Link Ambassador

What are the potential benefits for you and your healthcare team?

- Fewer non-medical primary care visits where the presenting symptoms include, for example, symptoms of loneliness and/or social isolation.
- Fills in gaps in the healthcare system by providing links to community resources

What are some of the anticipated benefits for your patients?

- Helps with social and community connections and expands the older adult's network of peer support
- Healthcare providers are able to use their time more effectively by directing clients to sources that can address their non-medical needs
- Improves health and well-being
- Potentially fewer visits to hospital emergency departments
- Barriers are reduced as older adults are empowered to identify their needs and values and to connect with what matters most to them

What does the research say?

According to a 2015 study of over 70 papers involving 3.4 million participants led by Brigham Young University, a lack of social connections is as great a risk factor for early death as smoking 15 cigarettes a day and constitutes a greater risk than obesity and lack of exercise. It is associated with increased risk of heart disease, stroke, depression and substance misuse leading to more frequent hospital admissions, longer stays in hospital and long-term care admissions (Holt-Lunstad J, Smith TB, Layton JB (2010) Social Relationships and Mortality Risk: A Meta-analytic Review. PLOS Medicine 7(7): e1000316. https://doi.org/10.1371/journal.pmed.1000316)

Additional resources

"If my doctor hadn't prescribed the exercise program, I would never have set foot in the Leisure Centre in the first place!"
……..Alex

1. Alliance for Healthier Communities (*Ontario's Social Prescribing Pilot Project lead)

https://www.allianceon.org/Social-Prescribing

2. National Academy for Social Prescribing, U.K.

https://socialprescribingacademy.org.uk/

3. The Social Prescribing Network

https://www.socialprescribingnetwork.com/

4. Canadian Institute for Social Prescribing

https://www.socialprescribing.ca/

5. WHO Tool Kit on Implementing Social Prescribing

https://www.who.int/publications/i/item/9789290619765

6. National Institute on Ageing report *"Understanding Social Isolation and Loneliness Among Older Canadians and How to Address It"* https://www.niageing.ca/reports

ADD

YOUR CENTRE'S

LOGO HERE

Appendix J – Volunteer Role Postings

Volunteer Link Ambassador Role Posting

Position Title: Volunteer Link Ambassador (VLA)

Project: Links2Wellbeing: Social Prescribing for Older Adults

Goal of the project: Many older adults experience loneliness and/or social isolation. Participating in social activities can have a positive effect on their health and wellbeing. Social prescribing makes it easier for people to connect to others in their community. Social prescribing happens when a doctor, nurse practitioner or other PCP prescribes social activities for older adults who would benefit from more opportunities to meet others and to participate in activities.

The overall goal of this project is to increase the participation of older adults in social activities by connecting them to their local older adult centre on the recommendation (social prescription) of Primary Healthcare Providers (PCPs). These referrals may come to the centre from the Community Health Centre (CHC) in their community as well as from other PCPs such as Family Health Teams, solo physicians, community paramedics, etc. After the older adult is referred to the centre, the Volunteer Link Ambassador (VLA) will meet with him/her, complete the client intake form and help to connect the older adult to programs and services to suit their interests.

Time commitment and availability: Based on project needs, number of clients referred and may vary depending on whether the VLA is meeting with clients virtually or in person.

Location: *(please add your centre's address/location here)*

Responsibilities:

- Meet with the centre's designated employee/volunteer to understand who the client will be and their interests in being part of the centre.
- Meet with the potential client (online or by phone) to complete the client intake form and to explore specific areas of interest
- Match client with programs of interest and/or other services offered by the centre
- Follow up with the client at intervals outlined in the client intake form and potentially accompany them to a program (e.g., online, by phone and/or in person)
- Ongoing contact with the centre designate about the project and participant needs and interests
- Other duties as required

Knowledge and skills required:

- Comprehensive knowledge about the centre's programs and services
- Awareness and sensitivity of the diversity represented in the centre's membership

- Excellent oral, written and interpersonal communication skills
- Self-motivated and able to work independently
- Reliable with good organizational skills
- Good listening and interview skills to assist with matching older adults and programs (motivational interviewing skills an asset)
- Experience with computers and virtual applications (e.g., Zoom) is an asset

Contact information/Instructions on how to apply:

(each centre is to add instructions about how a volunteer can apply for a VLA position)

ADD

YOUR CENTRE'S

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social prescribing for older adults

Position Title: Volunteer Peer Mentors

Project: Links2Wellbeing: Social Prescribing for Older Adults

Goal of the project: Many older adults experience loneliness and/or social isolation. Participating in social activities can have a positive effect on their health and wellbeing. Social prescribing makes it easier for people to connect to others in their community. Social prescribing happens when a health care provider prescribes social activities for older adults who would benefit from more opportunities to meet others and to participate in activities. These individuals may be hesitant to participate in new programs and would do well to have the support of a designated person – a peer mentor – during their first few visits.

As a Volunteer Peer Mentor your role is to provide support to Links2Wellbeing referred clients in their first experience(s) at the centre. This could include:

- Leading individual tours or providing companionship during a group tour
- Sharing centre program, participation or membership information
- Introducing new members to class instructors and peers
- Attending a class (or two) with the new member so they feel comfortable and welcome (fees can be covered by the L2W subsidy fund)

Creating a welcoming environment to support the comfort, safety, and enjoyment of new participants is an integral part of ensuring new members feel a sense of belonging. This position offers a rewarding opportunity to engage with other seniors, build meaningful connections, make new friends and contribute to everyone's overall well-being.

Time commitment and availability: *(edit as per centre needs)* Out of pocket expenses will be covered i.e. program fees

Location: *(please add your centre's address/location here*)

Knowledge and skills required:

- Comprehensive knowledge about the centre's programs and services
- Reliable, self-motivated and able to work independently
- Excellent listening and communication skills: Possess strong verbal communication skills and the ability to communicate effectively with seniors, staff members, and fellow volunteers.
- Patience and Understanding: Exhibit patience, understanding, and tolerance when assisting seniors, particularly those with cognitive or physical limitations.
- Physical Stamina: Have the physical stamina to stand, walk, and assist seniors throughout a tour or exercise class *(may not be required for all mentor volunteers)*

- Respect for Confidentiality: Maintain confidentiality regarding sensitive information shared by seniors and adhere to ethical guidelines regarding privacy and confidentiality.
- Cultural Sensitivity: Demonstrate cultural sensitivity and awareness when interacting with seniors from diverse backgrounds and respecting their cultural beliefs and practices.
- Demonstrate a genuine concern for the well-being of Older Adults and a compassionate attitude towards their needs and challenges.

This volunteer position offers a valuable opportunity to make a meaningful difference in the lives of seniors by providing companionship, support, and assistance during centre tours and activities. We welcome individuals who are dedicated to enhancing the well-being and quality of life of older adults in our community.

Contact information/Instructions on how to apply:

(each centre is to add instructions about how a volunteer can apply for a Volunteer Friend position)

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Older Adult Centres' Association of Ontario - OACAO - Association des centres pour aînés de l'Ontario

Appendix K Volunteer Pledge & Confidentiality Agreement

As a Volunteer for the Links2Wellbeing project with _____ (*name of centre*), I may become aware of client private and confidential information. I agree that I will not disclose such information without the client's consent. At all times I will respect the privacy of clients, their families, other employees, and volunteers.

I will ensure private and confidential information is not inappropriately accessed, used, or disclosed either directly by me or by virtue of my access to electronic and/or paper system and records.

When a referral to a program is appropriate, I will ask for the client's permission before sharing their name, telephone number, e-mail, or any other information I might have about the client.

Some examples of breaching privacy and confidentiality may include but are not limited to:

- Accessing personal information that I do not require for work purposes.
- Misusing or disclosing personal information verbally, through the computer system or in hard copy, without proper authorization; and/or
- Altering personal information of clients, their families, other employees, and volunteers.

If I have any questions or concerns about confidentiality, I understand that I may contact _____. *(add DCC/staff name and contact information*).

I understand this confidentiality pledge remains in effect even if I cease to continue in my role as Volunteer Link Ambassador/Volunteer Peer Mentor.

I have read, understand, and agree with the above statements.

Date: _____

Volunteer signature: _____

Witnessed by: _____

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Appendix L: Year-End Financial Report

Year-End Financial Report

Organization Name: _____ Date: _____

Contact Name: _____ Email: _____

Income Section:
Column A: Please clearly indicate all funding carried forward from previous years. If you need to re-confirm this total prior to starting please email L2W@oacao.org
If this is your first year in the project leave this column blank.

Column B: This is all funding *approved and received* in the current reporting year. This should include any confirmed cheques that may be in the mail.

Expense Section:
The amount spent during this reporting period. It is advisable to account for spending of carry forward funding from previous years, prior to that of your current year.

Carrying forward funds: unspent funds will be reviewed and approved by OACAO upon submission of all year end reports.

Please ensure Client Information Form data is entered into SurveyMonkey as well.

Unspent funds not being carried forward must be returned to OACAO by April 30 of each reporting year. This can be done by a cheque to OACAO, P.O. Box 65, Caledon East, ON, L7C 3L8 or bank EFT.

….see next page for reporting form

INCOME	Funds Carried Forward from Previous year(s) (A)	Funding Received in current fiscal year (B)	Total Funds Available (A + B)
Subsidies (i.e. program fees, membership fees)	$	$	$
Project Expenses - Admin.(i.e printing, postage, volunteer screening)	$	$	$
Other Income if applicable. (donations, funding from other organizations etc)	$	$	$
TOTAL INCOME:	**$**	**$**	**$**
EXPENSES	**Amount Spent with carry forward funds (C)**	**Amount Spent with Current Funds (D)**	**Total Spent as of March 31 (C + D)**
Subsidies	$	$	$
Project Expenses – Admin	$	$	$
Other Expenses: if any, not covered by funding	$	$	$
TOTAL EXPENSES:	**$**	**$**	**$**
Balance / Carry Forward: (TOTAL INCOME minus TOTAL EXPENSES):			**$**

Financial Reporting Form

* **Request to carry forward Unspent Funds: Yes _____ No _____**
* **Amount Requested to Carry Forward (Balance): $ _____**

Appendix M: Outreach Postcard - for community organization and health care providers

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Attention Health Care Providers
Are You aware of your local Seniors Active Living Centre?

Do you have older adult patients or clients who are socially isolated and would benefit from social, recreation and physical programs?

The Older Adult Centres' Association of Ontario (OACAO), in partnership with the Alliance for Healthier Communities, is co-hosting the **Links2Wellbeing: Social Prescribing for Older Adults Project,** linking isolated seniors with their local seniors' centre through physicians, nurse practitioners and other allied health professionals.

For more information, call Toll Free 1-866-835-7693, email L2W@oacao.org or visit www.oacao.org/programs/links2wellbeing

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What is a Seniors Active Living Centre?
Seniors Active Living Centres offer social, cultural, learning and recreational programs for older adults that promote health, wellbeing and social connections.

What benefits come from participating in Links2Wellbeing?

- Enhanced emotional and/or physical wellbeing
- Increased feelings social inclusion, decreased feelings of loneliness
- Make new social connections and friends
- Removing barriers to participation (such as providing membership/ program subsidies or supplies, transportation costs if available)

To find a participating Seniors Active Living Centres (SALCs) near you visit:
https://www.socialprescribingquilt.com

To learn more about the benefits of social prescribing, scan here:

Appendix N – Information Postcard for Older Adults

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social prescribing for older adults

Has your health care provider suggested a social prescription?

What is Social Prescribing?

Social prescribing allows healthcare providers to write a "prescription" to connect older adults with programs and services in the community that improve their social, mental, and physical health and wellbeing.

Once "prescribed" a referral will be sent to the Seniors Active Living Centre closest to you. Soon after, a centre contact will connect with you to share information about the activities and services they provide and invite you for a tour.

What is a Seniors Active Living Centre?

Seniors Active Living Centres offer social, cultural, learning and recreational programs for older adults that promote health, wellbeing and social connections.

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What benefits come from participating in Links2Wellbeing?

- New social connections and friends, participate in new skills and activities
- Increased feelings of social inclusion, decreased feelings of loneliness
- Increased opportunities for physical activity and wellness
- Enhanced emotional wellbeing, better mental health

The Links2Wellbeing program works to reduce barriers to participation. Participating in Links2Wellbeing provides access to membership and program subsidies, transportation support (where available), along with supportive staff and volunteers.

To learn more about the benefits of social prescribing, scan here:

For more information
Phone: 1-866-835-7693
Email: L2W@oacao.org
Visit: oacao.org/programs/links2wellbeing/

Appendix O: Promotional Poster for Referral Partners

Older Adults are Building New Connections and Friendships in Your Community!

The Links2Wellbeing: Social Prescribing for Older Adults project can help:

- Improve health and wellbeing by connecting to social, recreational and physical programs
- Enhance access to in-person and virtual programs
- Subsidize memberships or transportation fees (where available)

For more information
1-866-835-7693
L2W@oacao.org
oacao.org/programs/links2wellbeing/

Scan to find a participating Seniors Active Living Centre near you

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INSERT YOUR LOGO HERE

Appendix P: Outreach Cover Letter

Links2Wellbeing: Social Prescribing for Older Adults

In an effort to help reduce social isolation and loneliness within the older adult community in *insert town/city*, we are reaching out to you to share some information about an exciting project we are participating in called **Links2Wellbeing: Social Prescribing for Older Adults (L2W)** co-led by the Older Adult Centres' Association of Ontario (OACAO) and the Alliance for Healthier Communities.

Social prescribing is a means for healthcare providers to use a formalized pathway of referral to connect patients to a range of non-clinical programs and services in the community, to improve their social, mental, and physical health and wellbeing.

Insert your centres name offers quality programming and social opportunities for older adults. Programs include social activities, physical activities such as sports and recreation, arts and crafts, cards and games, educational activities, health, and wellness and more! Older Adults are involved with the program plan and can connect with social activities of their choice.

To refer a client/patient to the Centre, please complete the **referral form** attached and if available provide your patient with a **written Social Prescription** using the Social Prescription Pads provided by the Centre**.** Upon receipt of a Social Prescription Referral, or once we are contacted by the referred older adult, our Links2Wellbeing team lead will connect with the referred client and start the process of engaging collaboratively to co-create and match their areas of interest and goals to our programs and/or services offered by our Senior Active Living Centre in *town/city.* The project focuses on social inclusion and subsidies are available to remove barriers to participation.

Included with this package you will find:

- Information sheet about the L2W project from OACAO
- Social Prescription pads for your patients/clients
- Client referral form (also available digitally to upload to your electronic referral system)
- Information about our Seniors Active Living Centre *or add Centre Name here*

Feel free to contact us should you require more information or printed materials or wish to discuss this program further.

Kind Regards,

Name, position
Centre name
Contact information
Copy for accepting edits